

LEGISLATURE OF NEBRASKA  
ONE HUNDRED NINTH LEGISLATURE  
SECOND SESSION

**LEGISLATIVE BILL 777**

Introduced by Cavanaugh, M., 6.

Read first time January 07, 2026

Committee:

- 1 A BILL FOR AN ACT relating to the Medical Assistance Act; to amend
- 2 section 68-908, Revised Statutes Cumulative Supplement, 2024; to add
- 3 eligibility and reporting requirements for the Department of Health
- 4 and Human Services as prescribed; and to repeal the original
- 5 section.
- 6 Be it enacted by the people of the State of Nebraska,

1           **Section 1.** Section 68-908, Revised Statutes Cumulative Supplement,  
2   2024, is amended to read:

3           68-908 (1) The department shall administer the medical assistance  
4   program.

5           (2) The department may (a) enter into contracts and interagency  
6   agreements, (b) adopt and promulgate rules and regulations, (c) adopt fee  
7   schedules, (d) apply for and implement waivers and managed care plans for  
8   services for eligible recipients, including services under the Nebraska  
9   Behavioral Health Services Act, and (e) perform such other activities as  
10   necessary and appropriate to carry out its duties under the Medical  
11   Assistance Act. A covered item or service as described in section 68-911  
12   that is furnished through a school-based health center, furnished by a  
13   provider, and furnished under a managed care plan pursuant to a waiver  
14   does not require prior consultation or referral by a patient's primary  
15   care physician to be covered. Any federally qualified health center  
16   providing services as a sponsoring facility of a school-based health  
17   center shall be reimbursed for such services provided at a school-based  
18   health center at the federally qualified health center reimbursement  
19   rate.

20           (3) The department shall maintain the confidentiality of information  
21   regarding applicants for or recipients of medical assistance and such  
22   information shall only be used for purposes related to administration of  
23   the medical assistance program and the provision of such assistance or as  
24   otherwise permitted by federal law.

25           (4) The department shall provide the maximum amount of retroactive  
26   coverage for each medical assistance eligibility category as permitted by  
27   federal law.

28           (5) ~~(4)~~ The department shall prepare an annual summary and analysis  
29   of the medical assistance program for legislative and public review. The  
30   department shall submit a report of such summary and analysis to the  
31   Governor and the Legislature electronically no later than December 1 of

1 each year. The annual summary shall include, but not be limited to:

2 (a) The number and percentage of applications approved and denied;

3 (b) The number of eligibility determinations, including the number  
4 and percentage of those individuals remaining enrolled, terminations, and  
5 other determinations;

6 (c) The number of case closures in the medical assistance program  
7 and the Children's Health Insurance Program and the specific reason for  
8 the closure broken down by (i) eligibility category, including program  
9 type, (ii) local public health district or other geographic area, and  
10 (iii) race or ethnicity, if available;

11 (d) The number of medical assistance program and Children's Health  
12 Insurance Program enrollees broken down by (i) eligibility category,  
13 including program type, (ii) local public health district or other  
14 geographic area, and (iii) race or ethnicity, if available;

15 (e) The number and percentage of redeterminations or renewals  
16 processed ex parte, broken down by (i) eligibility category, including  
17 program type and (ii) race or ethnicity, if available;

18 (f) The average number of days required to process applications for  
19 the medical assistance program and Children's Health Insurance Program,  
20 separating the data by applicants with modified adjusted gross income and  
21 nonmodified adjusted gross income eligibility;

22 (g) The rate of re-enrollment within ninety days of termination and  
23 within twelve months of termination, broken down by (i) eligibility  
24 category, including program type, (ii) local public health district or  
25 other geographic area, and (iii) race or ethnicity, if available;

26 (h) The average client call duration;

27 (i) The client call abandonment rate;

28 (j) The number of requests for a fair hearing separated by (i)  
29 eligibility category and program type, (ii) outcome, and (iii) amount of  
30 time until final disposition; and

31 (k) A link to the medical assistance program fair hearing decisions

1 that have been redacted to protect private and health information, which  
2 shall be posted on the department's website; -

3 (1) The status of community engagement requirements, including:

4 (i) A description of the plans to implement community engagement  
5 requirements for medicaid recipients, including the authority and  
6 effective date for the requirements and the recipients subject to the  
7 requirements;

8 (ii) The number of denied applications and renewals for failure to  
9 meet community engagement requirements;

10 (iii) The number of applications and renewals denied because the  
11 community engagement requirement verification could not be completed;

12 (iv) The number of applications and renewals which required the  
13 recipient to submit additional information relating to compliance with  
14 community engagement requirements;

15 (v) The number of applications and renewals approved because the  
16 applications and renewals received an exemption, the type of exemption,  
17 whether or not the exemption was applied automatically, and whether or  
18 not the recipient was required to take action to receive the exemption;

19 (vi) The number of applications and renewals approved because the  
20 applications and renewals complied with the community engagement  
21 requirement, disaggregated by the compliance activity type, whether or  
22 not compliance was determined automatically, and whether or not the  
23 recipient was required to take further action in order to be approved;

24 (vii) The number of applications and renewals denied or terminated  
25 due to a failure to meet community engagement requirements in which the  
26 recipient was re-enrolled within ninety days and the number of such  
27 applications and renewals in which the recipient was re-enrolled within  
28 twelve months;

29 (viii) A list of data sources the department uses to verify  
30 compliance or exemption status; and

31 (ix) A list of external vendors contracted by the state to assess

1 compliance with, or exemption from, community engagement requirements,  
2 including a link to each vendor's current contract;

3 (m) The number of identified cases of concurrent enrollment and  
4 external vendors contracted by the state to identify concurrent  
5 enrollees, including a link to each vendor's contract. For cases  
6 terminated for concurrent enrollment, the rate of re-enrollment within  
7 ninety days after the date of termination and the rate of re-enrollment  
8 within twelve months after the date of termination; and

9 (n) A description of cost sharing, premiums, copays, and deductibles  
10 for goods and services provided under the medical assistance program,  
11 including (i) the amounts of the cost sharing, premiums, copays, and  
12 deductibles and (ii) the payment source for collected cost sharing.

13 **Sec. 2.** Original section 68-908, Revised Statutes Cumulative  
14 Supplement, 2024, is repealed.